



PROCEDURE

Emergency Management On Call

Scope (Staff):	All NMHS Mental Health Executive Rostered On Call
Scope (Area):	NMHS Mental Health

Contents

1. Aim.....	1
2. Background.....	1
3. Risk.....	2
4. Definitions	2
5. Key Points.....	2
5.1 Aishwarya's CARE Call (ACC)	3
5.2 Executive On Call Contact Number	3
6. Executive On Call Role	3
6.1 Executive On Call Process	3
6.2 Executive On Call – Bed Management.....	4
6.3 Executive On call – Public Relations	4
7. Coordinator of Nursing Role.....	5
7.1 Coordinator of Nursing – Responding to Clinical Issues.....	5
7.2 Coordinator of Nursing – Bed Management	6
8. Training Requirements to Undertake Role	6
9. Rumeration for On Call Period	6
9.1 Awards and Agreements	6
10. Reporting	6

1. Aim

This procedure provides governance for Mental Health Services (MHS) relating to issues and incidents which occur after hours.

2. Background

MHS provides services that range from Monday – Friday during business hours to inpatient patient care which is 24/7. MHS recognises the importance of supporting critical services outside of the core business hours including after-hours weekends and public holidays.

This procedure provides a framework to ensure:

- Services that operate 24/7 are adequately supported with an effective response and resolution provided to matters requiring urgent attention after hours

- A consistent approach to the implementation of on call and recall arrangements and,
- Employees who may be required to provide an essential on call and recall service, in order to respond to services needs and to operational service requirements, are remunerated correctly .

On call staff are scheduled as per the MH Emergency Management Roster, published monthly. The MH Emergency Management Roster identifies nominated roles that undertake emergency functions.

3. Risk

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☒	Policy & Legislative Compliance	☐
Fiscal Responsibility	☒	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Definitions

On call	An employee is on call when directed by the employer to remain at a place that will enable the employer to readily contact the employee during the hours when the employee is not otherwise on duty.
Recall (requiring attendance)	An employee, other than one accommodated at the Hospital, who is recalled to work for any purpose will be paid a minimum of three hours at the applicable overtime rate. The employee will not be obliged to work for three hours if the work for which the employee was recalled is completed in less time.
Recall (not requiring attendance)	Where an employee rostered on call by the employer is recalled to duty for the purposes of that on call roster; and (i) is recalled to duty for the purposes of that on call roster; and (ii) it is agreed the employee is able to undertake all the required work without attending their normal workplace or another work location designated by the employer; the employee will be paid for a minimum of one hour, or for the actual time worked if in excess of one hour, at the applicable overtime rate.

5. Key Points

- Rostered staff on the MH Emergency Management On Call Roster:

- Are expected to be contactable at all times via mobile and available to respond to requests.
- Must be within one hour of Perth metropolitan area, to respond to services in person if required.
- Must be familiar with the sites and services within MH and their core business.
- Must be familiar with and able to access State, North Metropolitan Health Service (NMHS) and MHPHDS policies.
- Have knowledge of role and accountabilities for emergency response
- Have sound understanding of MH patient flow Bed management principles

5.1 Aishwarya's CARE Call (ACC)

WA Health has directed all Hospital Service Providers (HSP) to establish a system-wide process to respond to concerns from patients, family and carers. ACC is the public facing name of this escalation process.

MHPHDS has established a process for responding to ACC.

Patients, family or carers can make an ACC from their mobile phone by calling **1800 792 621**.

- The Executive On call is the first contact point all hours for ACC 24/7 during the period of on call.
- All calls and associated actions need to be logged in the on call report.
- Services need to be advised as required and any actions must be followed up.
- If required, actions must be recorded in the patients file by clinicians and added to the weekly on call report.

5.2 Executive On Call Contact Number

A generic mobile number (0422 148 483) is available for NMHS MH or other HSPs to contact the Executive on call if they are unaware who is on call. The number is diverted weekly on a Monday to the incoming staff member assuming Executive On Call role. This process is coordinated by the Emergency Management Unit.

6. Executive On Call Role

The Executive On Call:

- Has afterhours oversight of MHS.
- Provides afterhours managerial assistance for urgent issues.
- Is the first contact point all hours for ACC, as noted above.
- Undertakes the role of Hospital Incident Commander (HIC) in the event the Incident Management Team (IMT) is activated.

6.1 Executive On Call Process

The Executive On Call should be contacted regarding:

- Any concern/uncertainty requiring advice or support from the Coordinator of Nursing and On Call Psychiatrists from inpatient units across NMHS MH;
- Infection/outbreak in clinical areas;

- Code Yellow internal emergency;
- Code Black of significance (injury to patients or staff);
- Patient Absent Without Official Leave (AWOL) (high profile/with potential media interest);
- Activation of the Emergency Codes and /or Business Continuity Plan (BCP), including unplanned paging/telephone /security systems/duress downtime;
- Serious illness or an accident affecting any staff member at work;
- Serious complaints involving any staff member or area;
- Any deaths in the inpatient unit of a patient (in particular those requiring Coroner investigation e.g. patients under the MHA);
- Industrial action or staffing deficits affecting delivery of service;
- Issues notifiable to the Chief Psychiatrist, WA; and
- Any incident with potential media interest/involvement.

The Executive On Call should consider attending the service should there be a serious incident such as the unexpected death of a patient or major staff incident in order to provide Executive oversight and incident support.

6.2 Executive On Call – Bed Management

The Executive On Call is responsible for supporting the On Call Psychiatrists and the clinical areas to ensure that they meet local bed capacity demands and maintain the quality and safety of patient care.

- For each inpatient service, the rostered On Call Psychiatrist is accountable to work collaboratively with NMHS Patient Flow team to facilitate patient admissions and discharges.
- The On Call Psychiatrists are to assist the Emergency Department (ED) when capacity is reached/bypass/ramping and address patients of concern in ED with respect to either length of stay in ED or lack of a MH management plan.
- The Statewide Patient Flow Medical Director On Call will provide clinical opinion and will make final decisions on the prioritisation of beds following consultation with the Executive On Call to resolve ED capacity issues.

6.3 Executive On call – Public Relations

Any communication with the media must be cleared by the Executive Director, MHPHDS and NMHS Corporate Communications team. They can be contacted through NMHS Communications Switchboard (6159 6600).

Executive On Call must escalate any **significant** issues and or concerns by exception outlining the operational contingency plans to the Executive Director, MHPHDS

This includes:

- Severity Assessment Code (SAC) 1 clinical incidents
- Public relation issues
- Emergency Codes
- COVID exposure within one of our services

Dependent upon the severity of the issue, the escalation may occur either via telephone call, SMS message or email.

For incidents of concern, the Executive On Call will have a discussion and email with the relevant Executive of the impact service the next business morning to appraise them of the issue and any ongoing actions.

7. Coordinator of Nursing Role

The Coordinator of Nursing is rostered after hours to provide governance, leadership, consultation, support and guidance to nurses for complex clinical care and critical incidents in all inpatient and community MHS in MH.

- Within each MHS, a senior nurse is designated as the point of contact to respond to queries or concerns raised, noting within Inpatient this would sit with the CNS
- If the senior nurse is unable to resolve issues raised within the service or requires support, the senior nurse will escalate the matter to the Coordinator of Nursing on Call.
- Clinical issues if unresolved with local resources can also be escalated to the On Call Psychiatrist to support the clinical team.
- The Coordinator of Nursing ensures that the Executive On Call is kept informed of issues of concern or incidents requiring further escalation.

7.1 Coordinator of Nursing – Responding to Clinical Issues

The senior nurse (CNS/CNM) will consult with the Coordinator of Nursing on complex issues to assist in decision making and actions or for relevant approvals.

- In the first instance, the Coordinator of Nursing will endeavour to resolve the issue via telephone. This may include consultation with medical staff or the Executive On Call.
- The senior nurse is to document the reason for the contact and outcome (e.g. patient's medical record, shift report).

The Coordinator of Nursing will attend the service in the event of:

- Complex clinical care that cannot be managed by the nursing and medical staff on duty or where the situation requires direct oversight and support;
- A patient death requiring an investigation by the Coroner; (applies to inpatient units)
- Significant injury to patients or staff;
- Activation of the Incident Management Team (IMT);

The senior nurse in the service will contact the Coordinator of Nursing in the event of:

- Any concern/uncertainty requiring advice or support from senior nurses in inpatient or community sites.
- All SAC1 incidents and any serious incidents involving staff
- Infection outbreak in clinical areas or IPC admissions needing advice
- Code Yellow internal emergency
- Code Black of significance (injury to patients or staff)
- Activation of the Emergency Codes and /or BCP - including unplanned paging/telephone /security systems /duress downtime
- Serious illness or an accident affecting any staff member at work
- Serious complaints involving any staff member or area
- Any death requiring investigation by the Coroner in an inpatient unit
- Industrial action or staffing shortages affecting delivery of service

- Any incident with potential media interest/involvement or potential reputational damage for the service.

The Coordinator of Nursing shall report all issues of concern or incidents requiring further escalation to the Executive On Call (e.g. SAC 1 clinical incidents, public relation issues and relevant Emergency Code activations).

7.2 Coordinator of Nursing – Bed Management

The Coordinator of Nursing liaises with the senior nurse and the On Call Psychiatrist on issues relating to after-hours bed management such as bed capacity and staffing resources to meet local bed capacity demands and maintain the quality and safety of patient care.

8. Training Requirements to Undertake Role

Executive On Call and Coordinator of Nursing must undertake MH Emergency Management On Call training prior to commencing on call activities and should ensure they are familiar with all sites to assist in decision making. Training is provided by the Emergency Manager.

9. Remuneration for On Call Period

The Executive On Call and Coordinator of Nursing will submit the HSS P7 form to their relevant line manager for approval following completion of the on call week including any recall or overtime conducted.

Please refer to [definitions](#) in relation to on call vs recall.

9.1 Awards and Agreements

Area	Link to the award or agreement
Medical Award	Medical
Nursing Award	Nursing
HSU Award	Salaried Officers

10. Reporting

The Executive On Call and Coordinator of Nursing is responsible for ensuring that any events, issues or incidents are recorded in the online MH On Call Weekly Report which is completed on REDCap within 3 days of your on call completion.

<https://datalibrary-rc.health.wa.gov.au/surveys/?s=998RRCTWPKE39AFX>

Related internal policies, procedures and guidelines
NMHS Clinical Incident Management Policy NMHS Communications Policy

References

[Mental Health Bed Access, Capacity and Escalation – Statewide Policy](#)
[PHEOC Guidelines for Management and Contact Tracing of COVID-19 with Exposure in the Hospital Setting](#)

Useful resources

List and hyperlink the titles of useful resources, do not hyperlink MR forms

Policy Sponsor	Executive Director MHPHDS				
Policy Contact	Emergency Manager MHPHDS				
Date First Issued:	08/01/2025	Last Reviewed:	08/01/2025	Review Date:	08/01/2028
Approved by:	Emergency Manager MHPHDS			Date:	08/01/2025
Endorsed by:	MHPHDS Executive Director			Date:	08/11/2024
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☐  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety		☐  Std 5: Comprehensive Care ☐  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration		
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Document Version Control			
Date	Version	Author	Notes
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